Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°150 /MVE-BDBV/11/10/2026

Report date: October 12, 2026 Publication date: October 13, 2026

HIGHLIGHTS (last 24 hours)

- **Eighty-eight (88) new confirmed cases of Ebola Virus Disease (EVD), including 37 community deaths, have been reported. These cases originated from the provinces of Ituri (56 cases), North Kivi (28 cases), and Haut-Uele (4). In addition, 8 deaths were recorded in the CTEs.**
- **A total of 423 new samples were analyzed; 88 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 20.8%.**

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9065	4 381	48,3 %	895	2 389	79,5%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 Health Zones affected out of 167 (38,9%)

Summary of key points from the last 24 hours

As of October 11, 2026, **88 new confirmed cases** of Ebola virus disease (EVD) were reported in three provinces: **Ituri (56 cases), North Kivu (28 cases) and Haut-Uélé (4 cases).**

In addition, **8** deaths were recorded in the CTEs (3 in Haut Uele, 3 in Ituri and 2 in North Kivu), bringing the total number of confirmed deaths for the period in question to 45.

In addition, 33 patients (16 in Ituri, 10 in North Kivu, 5 **in Haut-Uélé and 2 in Tshopo)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded **9,066** confirmed cases and **4,381** deaths, representing a case fatality rate of **48.3%.** Ituri remains the epicenter of the epidemic, accounting for **73.8** % of cumulative confirmed cases and **63.6%** of new confirmed cases reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 11, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	56	6695	3 099	46,3%	28/36 (77,8 %)
North Kivu	28	1911	1 092	57,1%	17/34 (50,0 %)
Haut-Uélé	4	382	162	42,6%	7/13 (53,8 %)
Boss	0	59	22	37,3%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**88**	**9 065**	**4 381**	**48,3%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 11, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6695**	**3099**	**46,3%**	**56**	**22**	**3**	**25**
Give	12	1	8,3%				0
Understand	9	6	66,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	214	47	22,0%				0
have	2	2	100,0%				0
Bunia	1832	612	33,4%	20	7	1	8
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%	2	2	0	2
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	179	107	59,8%	1	0	0	0
Liter	302	154	51,0%	4	3	0	3
Logo	14	7	50,0%				0
Fight	35	9	25,7%				0
distributed	4	1	25,0%				0
Washing	43	18	41,9%	2	1	0	1
Darkness	258	132	51,2%	18	7	1	8
Mangala	370	161	43,5%	1	0	1	1
Mongbwalu	712	341	47,9%				0
Nanny	327	183	56,0%	3	2	0	2
Nizi	813	290	35,7%	4	0	0	0
Legumes	133	39	29,3%				0
Jungle	18	5	27,8%				0

Machine Translated by Google

Rwampara	1132	393	34,7%	1	0	0	0
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1911**	**1092**	**57,1%**	**28**	**15**	**2**	**17**
Me	458	309	67,5%	16	10	1	11
Farm	23	7	30,4%				0
Butembo	297	185	62,3%				0
Ten	1	0	0,0%				0
Kalunguta	70	26	37,1%	2	0	0	0
Cut	628	383	61,0%	5	4	0	4
Source	3	2	66,7%				0
Kyondo	37	20	54,1%				0
Lubero	28	12	42,9%				0
Be anxious.	63	22	34,9%	1	0	1	1
Alimbong	5	3	60,0%				0
Manguredjipa	15	5	33,3%	1	0	0	0
Masereka	36	12	33,3%				0
The museums	143	69	48,3%				0
Mutwanga	23	4	17,4%				0
Oicha	42	20	47,6%	3	1	0	1
Vuh	39	13	33,3%				0
Upper Uele	**383**	**162**	**42,3%**	**4**	**0**	**3**	**3**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	46	45,1%		0	2	2
Lining	100	48	48,0%	2	0	1	1
Beans	5	2	40,0%	1	0	0	0
Ordinary	128	45	35,2%	1	0	0	0
Boss	**59**	**22**	**37,3%**	0	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	6	1	16,7%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	12	37,5%				0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie -Rukula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**9066**	**4381**	**48,3%**	**88**	**37**	**8**	**45**

Machine Translated by Google

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 11, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	90	4	94	7	4	83	0	11	5
Bas Uélé	83	2	85	12	2	71	0	14	0
Ituri	1 029	63	1092	96	63	933	0	159	81
North Kivu	998	43	1041	107	41	891	2	148	57
Sud Ubangi	80	2	82	0	0	80	2	0	0
Boss	109	0	109	2	0	107	0	2	0
Total General	**2 389**	**114**	**2 503**	**224**	**110**	**2 165**	**4**	**334**	**143**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

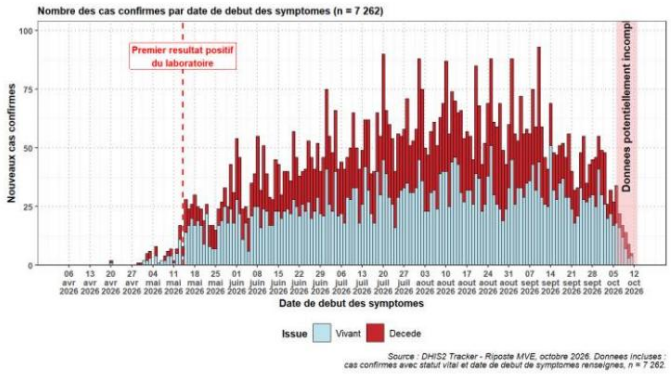
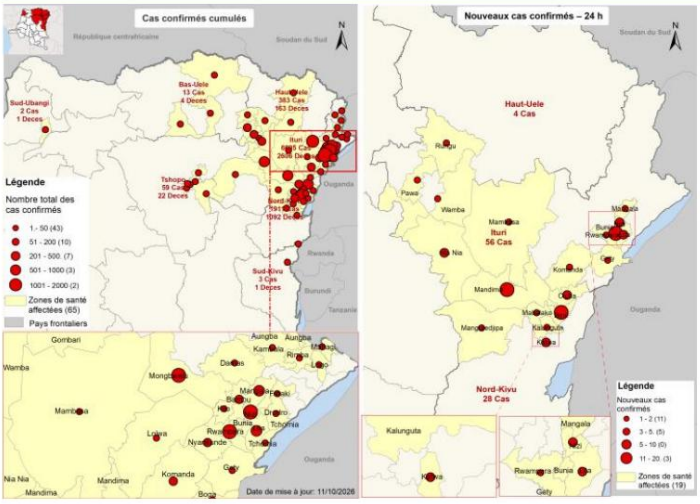


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

As of October 11, 2026, 2,503 alerts were reported in six provinces (Ituri, North Kivu, Haut-Uélé, Bas-Uélé, Sud-Ubangi, and Tshopo). All of these alerts were verified (100%), including 334 (13.3%).

Machine Translated by Google

validated as suspected cases. All 334 suspected cases were investigated (100%). Of these, 143 (42.8%) were transferred to CT/CTE.

Of the 27,043 contacts being monitored in the provinces that submitted their data, 21,512 were seen in the last 24 hours, representing a follow-up rate of 79.5%, down from the previous day (81.0%) and still below the required threshold of 95.0%. This rate was 100.0% (44/44) in Sud-Ubangi, 100.0% (321/321) in Bas-Uélé, 89.1% (1,558/1,749) in Haut-Uélé, 81.9% (6,746/8,236) in Ituri, 78.9% (276/350) in Tshopo, and 76.9% (12,567/16,343) in Nord-Kivu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Ten (10) alerts were notified to the PoE/PoC on October 11: six (6) in Ituri, three (3) in Bas-Uélé, and one (1) in Haut-Uélé; none in North Kivu or Tshopo. In Ituri, of the five (5) validated live alerts, only one was referred to the Komanda CTE (Yedi Base PoC) and tested positive, representing the 66th confirmed case detected at the PoCs; the other four refused transfer. In Bas-Uélé, a body was intercepted and swabed at the Bale PoC, and in Haut-Uélé, an alert from the Bombombi PoC was invalidated after testing.

Table 4. Status of key PoE/PoC indicators by province, as of October 11, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	98,3% (60/61)	95,2% (20/21) 100,0% (18/18)		98,0% (98/100)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	136 295	86 153	19 203	241 651
% of travelers screened	98,1%	97,6%	98,1%	97,9%
% of travelers who washed their hands	97,9%	97,6%	98,1%	97,8%
% of travelers made aware	99,5%	97,6%	100,0%	98,9%
Number of alerts notified	6	0	0	6
Number of validated live alerts (suspected cases)	5	0	0	5
Number of bodies recovered today	0	0	0	0
% of suspected cases transferred to CTE/CT	20,0%	THAT	THAT	20,0%
% of lifeless bodies swabed/secured	THAT	THAT	THAT	THAT
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	100,0%

The data for Haut-Uele and Bas-Uele are not included in the table.

2.2.2.

2.2.3. Challenges

3. In Ituri, the delay in payment of September salaries, the low encoding of alerts in the DHIS2 Tracker, and the refusal to transfer 4 out of 5 validated alerts persist; in North Kivu, the Butuhe PoC remains non-functional since 26/09/2026. In Haut-Uélé, 3 out of 10 PoCs did not report due to lack of connection, and in Bas-Uélé only 19 out of 41 notification sites are functional (46.3%).

3.1. Laboratory

3.1.1. Main actions

- **Ituri: 56 new positive results** (34 living and 22 deceased) out of 239 samples received and analyzed **(positivity: 23.4%).**
- **North Kivu: 28 new positive results** (13 living and 15 dead) out of 132 samples received and analyzed **(positivity: 21.2%).**
- **Haut-Uélé: 4 new positive results** (4 live) **out of 36 samples received and analyzed (positivity: 11.1%).**

Machine Translated by Google

- **Tshopo: 5 new** samples received **and analyzed (all live),** all tested negative
- **Bas Uélé: 11 new** samples received and analyzed **(all live),** all tested negative

3.2. Infection Prevention and Control (IPC/EDS)

3.2.1. Main actions

- **Ituri:** 8 out of 22 rings were opened (36.4%), 21 out of 25 households were decontaminated (84.0%), and 3 FOSAs Decontaminated out of 3 (100%); no households, social and solidarity economy establishments (ESS) or schools were provided with kits. Of 68 EDS alerts (including 15 postponements from the previous day), 55 EDS were carried out (80.9%) and 56 bodies were swabs.

- **North Kivu:** 23 rings were opened out of 32 (71.9%), with a gap in Katwa and Mabalako, 26 ESS decontaminated out of 32 (81.3%) and 14 households out of 30 (46.7%), none of the decontaminated households having been provided with kits; 4 PCI assessments (score card) were carried out in 4 ESS with an average score of 30.8%, 695 providers were briefed and no PPL were infected among the 25 confirmed cases of October 10 (0/25).
  Of 54 death alerts and 48 reports from the previous day, 57 EDS were carried out and 54 bodies swabed without failure, with 45 reports to the following day.

- **Haut-Uélé:** 2 households were decontaminated in Isiro (2/2; 100%), 2 ESS (Emergency Situations) in Wamba were monitored with briefings for 5 nurses on the donning and doffing of PPE, and 8 PPL (Personal Protective Equipment) workers were briefed (5 in Wamba and 3 in Boma Mangbetu). Out of 9 EDS (Emergency Discharge Summary) alerts, 8 EDS were carried out (88.9%) and 5 bodies were swabbed, without failure, with one EDS refusal in Isiro.

- **Tshopo:** 9 out of 9 ESS (Emergency Health Services) were monitored and 116 service providers briefed; the Vita Nova Community Health Center, the household of the 55th case, the CUKis (Community Health Centers), and the Makiso-Kisangani General Referral Hospital (58th case) were decontaminated. The cumulative total reached 41 out of 41 rings open (100%) and 35 out of 44 households decontaminated (79.5%); out of 2 EDS (Emergency Health Services) alerts, 1 body was swabed.

3.2.2. Challenges

In Haut-Uélé, the low completeness of ZS reporting (4/7), the lack of PCI kits for ESS and households and the absence of computer kit for encoding pillar data persist.

In Ituri, the opening of the rings remains insufficient (36.4%), there are insufficient teams in Komanda, Mangala and Mandima, mobility is lacking in Komanda, ECUMER coverage, EDS inputs and means of transport (Kilo, Nia-Nia, Mandima, Damas, Mambasa and Fataki) are insufficient and 8 resistances to EDS were recorded in Mandima.

In Tshopo, the shortage of PCI inputs and the breakdown of the incinerator at the Makiso-Kisangani General Referral Hospital persist; only 2 out of 104 ESS (1.9%) exceed 80% in the PCI score.

In North Kivu, the gap in the opening of the rings in Katwa and Mabalako, the low decontamination of households (14/30; 46.7%) in Katwa, Butembo and Musienene, the lack of provision of kits to decontaminated households, an average score card of 30.8% (no ESS out of 4 above 80%) and the high-risk exposure of 4 out of 5 staff assessed persist.

3.2.3. Holistic care

3.2.4. Main actions

- **In Ituri, 76** new admissions (10 of which were in advanced stages) were recorded, compared to 71 discharges, including 16 recoveries. Thus, 337 patients are currently hospitalized, representing an occupancy rate of 33.2%, including 41 critically ill patients; 21 patients were evacuated by ambulance, and 337 out of 347 patients (97.1%) received a hot meal. The total number of recoveries in the province has reached 1,895.

- **In North Kivu,** 79 new admissions were recorded compared to 109 discharges, including 10 recoveries. Thus, 471 patients (including 67 children under 5 years old) are hospitalized: 330 in CTE/CTs for 374 beds, representing an occupancy rate of 88.2%, and 107 (22.7%) in ESSs which are not CTE/CTs; 5 patients are in critical condition (Kitatumba CTE and Oicha CT) and all 20 ambulance trips were successful.

- **In Haut-Uélé,** 7 new admissions (1 confirmed in Isiro and 6 suspected in Pawa and Wamba) were recorded, compared to 5 recoveries in Boma Mangbetu. Thus, 45 patients, representing an occupancy rate of 32.6%, including 2 confirmed severe or critical cases in Pawa and Wamba; 55 patients, 67 caregivers, and 70 people with disabilities received hot meals, and 6 patients were transported by ambulance.

- **In Tshopo,** 11 new admissions were recorded compared to 2 discharges, all recoveries. Thus, 15 Patients are hospitalized in 36 beds, representing an occupancy rate of 41.7%, with no patients in critical condition; the 54th confirmed case has still not been transferred to the Ebola Treatment Center (ETC). Since the beginning of the outbreak, 443 people have received the experimental treatment, including 41 confirmed cases, and 402 have received post-exposure prophylaxis.

Machine Translated by Google

- **In Bas-Uélé,** 12 new suspected cases were admitted to transit centers, with no discharges or new confirmed admissions; post-exposure care continued in Limba (Viadana Health Zone) and the 21-month confirmed child isolated in Dingila remains monitored.

3.2.5. Challenges

In Ituri, the low capacity of some CTE/CTs (Mambasa, Kilo, Lolwa, Mangala, Mandima and Biakato), the care of 41 critical patients, the lack of food for 10 patients in the Mandima health zone and the low reporting of ambulance movements remain worrying.

In North Kivu, the saturation of suspected beds (157/165; 95.2%), the care of 107 patients (24.5%) outside of CTE/CT and an ambulance response capacity limited to 50% (14/28) remain worrying.
In Haut-Uélé, poor network coverage in some health zones, demotivation among some staff, insufficient nutritional inputs, and inadequate water and electricity supplies to some treatment centers remain challenges to be addressed.

In Tshopo, the lack of resuscitation equipment at the CTE and of staff for post-exposure prophylaxis weakens the care provided.

In Bas-Uélé, the lack of appropriate nutritional care for isolated patients, of ambulances for the transfer of patients and of standardized CTE, still under construction at the HGR of Buta, remains a major operational challenge.

3.3. Vaccination

3.3.1. Main actions

- **In Haut-Uélé,** the cumulative total reached 399 vaccinated PPL out of 858 pre-registered (46.5%), including 210 women and 189 men; in Isiro, 220 people out of 313 pre-listed were vaccinated at the HGR site (70.3%).

- **In Ituri,** data collection for the study on the impact of the EVD epidemic on the use of sexual and reproductive health services continued in the health zones of Mongbwalu, Bunia, Rwampara, Tchomia and Nyarambe, as did qualitative and operational surveys and the Partners, Oracle and EBO-PEP clinical trials.

3.3.2. Challenges

In Ituri, vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.

In Bas-Uélé, the implementation of the UCCs is not effective and the validated micro-plan as well as the vaccination funds are still not available.

- **In North Kivu,** indicators of continuity of essential services were analyzed in the health zones of Butembo, Beni, Katwa and Musienene for the purpose of supervision.
- **In Bas-Uélé,** the Dakwa General Referral Hospital is partially operational, with 13 staff members present out of 36 planned (36.1%), a decrease in attendance and the absence of a water point and a waste area.

3.4. Continuity of Care

3.4.1. Main actions • In Tshopo,

the evaluation of the Cliniques des Anges de Kisangani (CAKIS) showed an effective triage area, an operational incinerator and systematic use of the partogram, but only 6 agents vaccinated against Ebola out of 21 (28.6%); the recovery plan of the CS RTNC was developed, and the monitoring of PPL at the CTM and the CTE as well as that of recovered patients with comorbidities at the HGR of Makiso-Kisangani continued.

- **In North Kivu,** the encoding of assessments of the continuity of essential health services in the health establishments of the Butembo health zone continued.
- **In Bas-Uélé,** no data has been transmitted by the continuity of care pillar in the last 24 hours.

3.5. Risk Communication and Community Engagement (RCCE)

3.5.1. Main actions

- **In Ituri,** 24,812 home visits reached 83,512 people; 1,002 educational talks brought together 15,563 people, 8 mass awareness campaigns reached 2,090 people, 4 community debates reached 158 people and 8 advocacy campaigns reached 17 authorities and leaders; 38 spots were broadcast on 36 radio stations and 9 programs were organized.
  In total, 460 community alerts, including 129 deaths, were reported; 70 out of 85 feedback requests (82.4%) and 8 out of 11 incidents (72.7%) were resolved within 48 hours; 859 contacts were pre-listed around 19 confirmed cases; and one Ebola survivor was reintegrated into the village of Lumumba (Mangusu health district, Komanda health zone), with awareness-raising among community leaders to combat stigma. The completeness rate of CREC reports was only 42.9% (12/28 health zones).

- **In Bas-Uélé,** a mass awareness session reached 839 people at the market and in 8 churches in the Dakwa health zone; an interactive program was broadcast on the Rubi community radio station in Buta; a healer

Machine Translated by Google

53 people in Viadana were made aware and 11 members of a family in Dakwa agreed to isolation and testing of a suspected case; all 5 community alerts notified were dealt with.

- **In Tshopo,** 26 mass awareness sessions reached 31,861 people, including 4,219 children, in 26 churches, and an educational talk was held for 30 people in Kabondo; 20 community alerts were notified and all addressed, the 2 feedback received were resolved (100%), including a rumor about treating Ebola with palm flowers, and CREC supported the follow-up of 12 contacts and the listing of 6 new contacts in Kabondo

- **In Haut-Uélé,** 5 mass awareness sessions reached 5,060 worshippers in the churches of Wamba and Isiro and 2 educational talks reached 32 motorcycle taxi drivers in Wamba, totaling 5,092 people; 5 community alerts, including 3 deaths, were notified in Wamba and Isiro, all 3 feedback received were resolved (100%), and 31 family members of a suspected death were sensitized to contact tracing, decontamination, and EDS in Isiro, while the Vice-Governor took part in intensifying awareness efforts in Wamba.

- **In North Kivu,** 6,644 home visits reached 19,419 people, 185 educational talks reached 3,673 people, 45 mass communications reached 7,967 people and 1 briefing reached 26 journalists; 266 community alerts were notified out of 650 expected (40.9%), 14 feedbacks out of 17 (82.4%) were resolved within 48 hours, CREC actors were deployed in Sunday worship services and masses, 215 parents were sensitized at the Mangina Institute and 55 contacts were listed with the support of CREC in Lubero and Manguredjipa.

3.6. Mental Health and Psychosocial Support (MHPSS)

3.6.1. Main actions

- **In Ituri,** 17 of the last 30 confirmed cases (56.7%), 83 confirmed cases under follow-up out of 101 (82.2%), 26 new suspected cases out of 53 (49.1%), and 135 suspected cases under follow-up out of 160 (84.4%) benefited from SMSPS interventions. The teams supported 252 caregivers and 72 people living with HIV/AIDS (PLHIV), monitored 11 out of 16 recoveries (68.8%) in the community and 13 out of 13 children in daycare, assisted 7 out of 12 separated and orphaned households, reintegrated 5 out of 14 recoveries, and announced 31 results, 17 of which were positive; 41 children are currently being monitored.

- **In North Kivu,** 23 of the last 25 confirmed cases (92.0%), 212 confirmed cases under follow-up, 191 new suspected cases, and 376 suspected cases under follow-up benefited from SMSPS interventions in the 9 health zones that reported cases (100%); the teams assisted 98 households and 25 households of separated and orphaned children (100%), supported 89 people living with HIV, monitored 35 children in the community and 6 in daycare, reintegrated 10 recovered individuals, monitored 12 recovered individuals in the community, and announced 122 results, including 25 positive results; 384 people benefited from psychoeducation with UNICEF support

- **In Tshopo,** 7 confirmed cases are currently being monitored out of 7 (100%), 1 new suspected case and 3 suspected cases are currently being monitored Follow-up benefited from SMSPS interventions; 242 people benefited from psychoeducation, 3 results, including 2 positive ones, were announced and 25 APS from Bafwasende were trained.

- **In Haut-Uélé,** 19 confirmed cases under follow-up, 4 new suspected cases and 30 suspected cases under follow-up benefited from SMSPS interventions in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 33 caregivers, 64 PPLs, 11 children separated in the community and 9 household members, supported 6 EDS and announced 18 results, including 6 positive ones; in Bas-Uélé, no SMSPS interventions were reported in the last 24 hours.

3.6.2. Challenges

- **In Ituri,** only 14 out of 28 affected health zones are covered (50.0%); inter-pillar collaboration needs to be strengthened for joint interventions around cases in Komanda and Nia-Nia and the identification of local actors in Mandima needs to continue.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- **In Tshopo,** there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

Machine Translated by Google

3.7. Logistics

3.7.1. Main actions.

Ensure the mobility of all field teams in the affected provinces; in North Kivu, the opening of the Musienene CTE is planned for October 15.

3.8. Security

3.9. Main actions

- **In North Kivu,** teams were escorted to the burial of a confirmed case at the Bundji cemetery, the decontamination of a household in the Kasanga health district and the Arche dispensary (Ngongolyo health district), the swab of a deceased person at the Horizon morgue, and a situation assessment of the Kambuli health district (Katwa) with partner FHI 360; a demonstration session on EDS practices was also organized in Kambuli with the risk mitigation team

3.10. PSEA (Prevention of Sexual Exploitation and Abuse)

3.10.1. Main actions

- **In Tshopo,** 85 actors from six pillars of the response (52 women and 33 men) were briefed on the PSEAH with the support of the WHO and signed the code of good conduct; 641 people were sensitized in Kabondo, Makiso-Kisangani and Mangobo.

- **In North Kivu,** 26 mental health pillar providers (18 men and 8 women) were briefed on the PSEAH in Butembo and 125 people were sensitized on the PSEA and the reporting numbers 151 and 495555 in Mabalako (15) and in the AS of Vungi and Taraja in Katwa (110).

3.10.2. Challenges

At the national level, only 2 out of 7 DPS (28.6%) reported their PSEA activities (North Kivu and Tshopo); in Tshopo, coverage remains low, with 7 out of 23 health zones covered and 85 staff trained out of 270 (31.5%), while the proportion of health zones aware of the reporting mechanism remains low in Ituri (36.1%; 13/36), North Kivu (17.6%; 6/34) and Haut-Uélé (46.2%; 6/13).

MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, there is active community transmission while sporadic notification is observed in Tshopo province.

the of the

2. In the last 24 hours.

Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (79.5% vs. 81.0%).

%) and deserves special attention in order to avoid uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities , with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

Machine Translated by Google

TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP
Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager
Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager
Dr. KOBBE NGWAMA John

Editorial Team
Professor Dr. MULANGU KATULUMBA Félix-Médard
Dr. KATUSI BISELENGE Jules
Mrs. WISE DEAD Marie
Dr. DIUR MANANASHI Floryan
Mr. Kamuina Kebela John

Contact :
www.insp.cd
contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd